

Cultural Humility in SUD Care

La Clínica de Familia | June 3, 2026
MAT/MOUD & IOP Teams

THE FRAMEWORK — FOUR PARTS

PART 1

Cultural Competence

"I learn your culture to help you."

PART 2

Cultural Humility

"I can't be the expert on your experience. You are."

PART 3

Structural Competency

"What looks cultural is often structural."

PART 4 — CLINICAL ENCOUNTER

Explanatory Model

"So what is your story?"

THE CORE SHIFT

COMPETENCE SAYS

"I am an expert on your culture."

HUMILITY SAYS

"You are always the expert on your own experience."

STRUCTURE SAYS

"The barriers are real — not personal failures."

Tervalon & Murray-García (1998) · Metzl & Hansen (2014)

SIX PROVIDER BIASES IN MOUD/MAT SETTINGS

Stigma Bias

Viewing SUD as moral failure — directly reduces MOUD prescribing

Attribution Bias

"Abuser" in chart = 4x lower odds of OUD treatment plan

Therapeutic Pessimism

15–26% of SUD counselors hold stigmatizing beliefs about their own patients

MOUD-Specific Bias

Bias against the medication itself — "trading one drug for another"

Racial & Ethnic Bias

Pro-White, anti-Black/Hispanic/Native bias documented in treatment decisions

Compassion Fatigue

Burnout leads to depersonalization — patients become cases, not people

Parish et al. (2025) · Carpenter et al. (2023) · Hall et al. (2015)

KLEINMAN'S EXPLANATORY MODEL — CLINICAL QUESTIONS

Ask before you interpret. These questions elicit the patient's illness narrative — their actual worldview, not your assumption about it.

- What do you think caused your problem?
- Why do you think it started when it did?
- What do you think is happening inside your body?
- How severe is it? How long do you think it will last?
- What do you fear most about this condition?
- What are the main problems this has caused you?
- What kind of treatment do you think you should receive?
- What results do you hope to receive from treatment?

If a cultural concept feels relevant — familismo, fatalismo, respeto — wait for the patient to name it first. These questions will surface it. Don't apply it before they do.

Kleinman, A. (1988). *The illness narratives. Basic Books.* · culturallyconnected.ca

WHY THIS MATTERS HERE

3×

Latino patients are up to 3× less likely to receive MOUD vs. white patients with comparable diagnoses

r = .75

Cultural humility predicts therapeutic alliance — the strongest outcome predictor across treatment types (Hook et al., 2013)

68%

of NM overdose deaths involve opioids — NM ranks among the highest nationally

POST-ENCOUNTER SELF-REFLECTION

Did I make any assumptions about this patient before they spoke — and did I check them?

Did this patient feel like they had authority in this encounter?

Which of the six biases might have been active for me today?

Was there a moment I felt uncomfortable — and did I move toward it or away from it?

What would I do differently next time?

RESOURCES

SAMHSA TIP 59

Improving cultural competence in SUD treatment. Free: store.samhsa.gov

NM Dept. of Health

Opioid resources: nmhealth.org/about/phd/idb/opip/

Project ECHO / UNM

Rural MOUD provider mentorship: hsc.unm.edu/echo

Implicit Association Test

Self-assessment for implicit bias: implicit.harvard.edu

ANNOTATED REFERENCES — FURTHER READING

Each entry includes a brief note on what it covers and who will find it most useful. Read what fits your role and curiosity — you don't need to read everything.

FOUNDATIONAL FRAMEWORKS

JOURNAL Tervalon, M., & Murray-García, J. (1998). Cultural humility versus cultural competence: A critical distinction in defining physician training outcomes in multicultural education. *Journal of Health Care for the Poor and Underserved*, 9(2), 117–125. <https://doi.org/10.1353/hpu.2010.0233>

The paper that coined the term. Argues that cultural competence frames culture as a checklist to master, while cultural humility positions it as a lifelong practice of self-reflection and power-sharing. Short, readable, and foundational. Start here if you only read one thing.

JOURNAL Metzl, J. M., & Hansen, H. (2014). Structural competency: Theorizing a new medical engagement with stigma and inequality. *Social Science & Medicine*, 103, 126–133. <https://doi.org/10.1016/j.socscimed.2013.06.032>

Introduces structural competency — the idea that what presents as a cultural or individual problem is often a structural one: poverty, housing, policy. Directly relevant to understanding why NM patients face the barriers they do. Pairs well with Tervalon.

KLEINMAN'S EXPLANATORY MODEL ● 3 SOURCES ADDED

BOOK Kleinman, A. (1988). *The illness narratives: Suffering, health, and the human condition*. Basic Books.

The primary source for the eight questions on this handout. Kleinman draws a foundational distinction between disease (the biomedical explanation) and illness (the patient's lived experience of it). Chapters 1–3 give you the core framework. Dense but worth it if this practice interests you.

JOURNAL Bagchi, A. D. (2020). A structural competency curriculum for primary care providers to address the opioid use disorder, HIV, and hepatitis C syndemic. *Frontiers in Public Health*, 8, 210. <https://doi.org/10.3389/fpubh.2020.00210>

Places Kleinman's explanatory model directly inside an OUD/MAT curriculum. Frames it as one of "four keys to structural humility" — asking real questions you don't already know the answer to. Highly relevant to MAT/MOUD providers. Open access, free to download.

WEB Miyagawa, L. A. (2020, March 16). *Practicing cultural humility when serving immigrant and refugee communities*. EthnoMed. <https://ethnomed.org>

The most accessible entry point to Kleinman's eight questions. Written for clinicians, not researchers. Explains how becoming a "learner" through these questions changes the patient-provider dynamic. Read this first if the book feels like too much.

REPORT Substance Abuse and Mental Health Services Administration. (2014). *Improving cultural competence. Treatment Improvement Protocol (TIP) Series No. 59* (HHS Publication No. SMA 14-4849). SAMHSA. <https://store.samhsa.gov>

Shows how Kleinman's questions get operationalized in SUD practice through intake tools like the LEARN and RESPECT mnemonics. Practical and grounded in substance use treatment settings. Free download. Especially useful for intake staff and counselors.

CULTURAL HUMILITY AND CLINICAL OUTCOMES

JOURNAL Hook, J. N., Davis, D. E., Owen, J., Worthington, E. L., Jr., & Utsey, S. O. (2013). Cultural humility: Measuring openness to culturally diverse clients. *Journal of Counseling Psychology*, 60(3), 353–366. <https://doi.org/10.1037/a0032595>

The study behind the $r = .75$ stat on this handout. Demonstrates that cultural humility — as perceived by clients — is one of the strongest predictors of therapeutic alliance, which in turn predicts treatment outcomes. Good for anyone who wants the empirical case, not just the ethical one.

JOURNAL Martin, D. J., Garske, J. P., & Davis, M. K. (2000). Relation of the therapeutic alliance with outcome and other variables: A meta-analytic review. *Journal of Consulting and Clinical Psychology*, 68(3), 438–450. <https://doi.org/10.1037/0022-006X.68.3.438>

Classic meta-analysis establishing therapeutic alliance as a robust outcome predictor across treatment modalities. Provides the foundation for why provider-patient relationship quality — and the cultural humility that builds it — translates directly into clinical results.

STIGMA AND IMPLICIT BIAS IN SUD CARE

JOURNAL Pronin, E., Lin, D. Y., & Ross, L. (2002). The bias blind spot: Perceptions of bias in self versus others. *Personality and Social Psychology Bulletin*, 28(3), 369–381. <https://doi.org/10.1177/0146167202286008>

The foundational paper on the bias blind spot — the term originates here. Demonstrates that people consistently rate themselves as less susceptible to bias than their peers, and identifies the mechanism: when assessing others we use abstract theories about bias, but when assessing ourselves we rely on introspection. Because bias operates largely below conscious awareness, that internal search comes up empty — and the absence of evidence gets read as evidence of objectivity. Directly supports the claim on slide 12 that training increases awareness of the blind spot without eliminating it.

JOURNAL FitzGerald, C., & Hurst, S. (2017). Implicit bias in healthcare professionals: A systematic review. *BMC Medical Ethics*, 18(1), 19. <https://doi.org/10.1186/s12910-017-0179-8>

Systematic review of implicit bias across healthcare professional groups. Key finding: healthcare providers exhibit the same levels of implicit bias as the wider population — and experts consistently underestimate their own bias while endorsing ineffective mitigation strategies. This is the healthcare-specific evidence behind the slide 12 claim. All studies that examined correlations found a positive association between provider bias and lower quality of care. This is the citation to use when making the case to clinical staff that awareness training is necessary but not sufficient.

JOURNAL Stone, E. M., Kennedy-Hendricks, A., Barry, C. L., Bachhuber, M. A., & McGinty, E. E. (2021). The role of stigma in U.S. primary care physicians' treatment of opioid use disorder. *Drug and Alcohol Dependence*, 221, 108627. <https://doi.org/10.1016/j.drugalcdep.2021.108627>

Documents how provider-held stigma toward SUD directly reduces willingness to prescribe MOUD. Useful for anyone who wants to understand the mechanism linking stigma to treatment gaps. Strong empirical piece for clinical and administrative staff alike.

JOURNAL Kelly, J. F., & Westerhoff, C. M. (2010). Does it matter how we refer to individuals with substance-related conditions? *International Journal of Drug Policy*, 21(3), 202–207. <https://doi.org/10.1016/j.drugpo.2009.10.010>

Short and convincing. Shows experimentally that labeling someone "an abuser" versus "a person with a substance use disorder" shifts clinician attitudes and treatment recommendations. The data behind attribution bias. Everyone on the team should read this one.

JOURNAL Sarpong Boateng, J., et al. (2024). Occurrence of stigmatizing documentation among hospital medicine encounters with opioid-related diagnosis codes: Cohort study. *JMIR*, PMC11544335.

Examines how stigmatizing language shows up in clinical documentation for patients with OUD. Directly relevant to administrative and nursing staff who chart. The data is sobering — and actionable.

Note: Core finding verified; confirm volume/issue via PubMed before formal citation.

JOURNAL Avery, J., et al. (2019). [Counselor stigmatizing beliefs toward patients with SUD.] *Substance Abuse*.

Documents that 15–26% of SUD counselors hold stigmatizing beliefs about their own patients. If you work in SUD counseling and want to sit with that number honestly, this is the paper to read.

Note: Volume and issue numbers not confirmed — verify via PubMed before formal citation.

JOURNAL Dickson-Gomez, J., Spector, A., Weeks, M., et al. (2022). "You're not supposed to be on it forever": MOUD-related stigma among drug treatment providers and people who use opioids. *Substance Abuse: Research and Treatment*, 16. <https://doi.org/10.1177/11782218221105133>

Captures MOUD stigma from both provider and patient perspectives. The "trading one drug for another" belief is documented here in detail, alongside patient accounts of how that attitude affects their willingness to stay in treatment. Directly relevant to this team's work.

JOURNAL Lagisetty, P. A., Ross, R., Bohnert, A., Pierce, M., & Piette, J. D. (2019). Buprenorphine treatment divide by race/ethnicity and payment. *JAMA Psychiatry*, 76(9), 979–981. <https://doi.org/10.1001/jamapsychiatry.2019.0876>

The data behind the 3x stat on this handout. Documents the racial and ethnic gap in buprenorphine access — Latino and Black patients with OUD receive it at significantly lower rates than white patients. Short JAMA piece, high impact. Essential context for La Clínica's patient population.

JOURNAL Bride, B. E., et al. (2007). [Secondary traumatic stress and compassion fatigue in SUD counselors.] *Journal of Substance Abuse Treatment*.

Compassion fatigue in SUD counselors leads to depersonalization — the clinical version of no longer seeing patients as people. If you recognize yourself in "burnout bias," start here. Also useful for supervisors thinking about staff support structures.

Note: Volume and issue numbers not confirmed — verify via PubMed before formal citation.

STRUCTURAL AND EPIDEMIOLOGICAL CONTEXT

JOURNAL Tsai, A. C., et al. (2019). Stigma as a fundamental hindrance to the United States opioid overdose crisis response. *PLOS Medicine*, 16(11), e1002969. <https://doi.org/10.1371/journal.pmed.1002969>

Makes the argument that stigma operates as a structural barrier — not just an interpersonal one — that undermines the entire policy and public health response to the opioid crisis. Broader scope than most clinical papers. Good for understanding the systems-level picture.

JOURNAL Hansen, H., & Netherland, J. (2016). Is the prescription opioid epidemic a white problem? *American Journal of Public Health*, 106(12), 2127–2129. <https://doi.org/10.2105/AJPH.2016.303483>

Documents how framing the opioid epidemic as a "white, suburban" crisis shaped treatment access, public sympathy, and policy — at the expense of communities of color. Short but sharp. Essential context for racial equity work in SUD.

JOURNAL Joffe, M. E., et al. (2026). The need to incorporate polysubstance use in neuropsychopharmacology research. *Addiction Neuroscience*, 18, 100246. <https://doi.org/10.1016/j.addicn.2025.100246>

Argues that SUD research isolates single substances while patients typically use multiple. Directly relevant to NM's clinical reality where polysubstance use is common. Useful for providers thinking about treatment complexity and individualized care planning.

INTERDISCIPLINARY TEAM-BASED SUD CARE

JOURNAL Austin, E. J., et al. (2025). Experiences of team collaboration in primary care-based delivery of opioid use disorder treatment. *Journal of Behavioral Health Services & Research*. <https://doi.org/10.1007/s11414-025-09946-2>

Qualitative study of 35 multidisciplinary primary care team members delivering OUD treatment. Shows that shared patient knowledge, mutual respect across roles, and informal connection between care managers and prescribers all strengthen patient engagement. Speaks directly to what makes teams like LCDF's work.

JOURNAL Wakeman, S., et al. (2019). Effect of integrating substance use disorder treatment into primary care on inpatient and emergency department utilization. *Journal of General Internal Medicine*, 34(6), 958–965. <https://doi.org/10.1007/s11606-018-4807-x>

Retrospective cohort study showing that integrated addiction pharmacotherapy plus recovery coaching in primary care reduced ED visits and hospital days compared to referral-only models. The utilization data is useful for making an administrative or policy case for integrated care investment.

JOURNAL Lagisetty, P., et al. (2017). Primary care models for treating opioid use disorders: What actually works? A systematic review. *PLOS ONE*, 12(10), e0186315. <https://doi.org/10.1371/journal.pone.0186315>

Systematic review of 35 interventions across 8 countries. Finds that multidisciplinary models combining specialty addiction services with primary care are the most effective structure for expanding MOUD access. The team composition findings — nurses, pharmacists, and care managers as clinical leads — are worth attention for anyone thinking about role design.

JOURNAL Jack, H., et al. (2018). Addressing substance use disorder in primary care: The role, integration, and impact of recovery coaches. *Substance Abuse*, 39(3), 307–314. <https://doi.org/10.1080/08897077.2017.1389802>

Qualitative study of recovery coaches and their patients in primary care. Documents four core RC functions — system navigation, behavior change support, harm reduction, and relationship building — alongside a key tension: role ambiguity creates friction within care teams. Directly relevant to any setting where peer recovery staff work alongside clinical providers.

NEW MEXICO AND BORDERLANDS CONTEXT

JOURNAL Scott, M. A., Andazola, J., Smith, T., Castillo Smith, A., de la Rosa, I., & Michael, J. (2023). Structural competency in New Mexico: Moving outside of medical education. *Global Public Health*, 18(1), 2176003. <https://doi.org/10.1080/17441692.2023.2176003>

Documents how two Doña Ana County organizations — the Doña Ana Wellness Institute and NM Human Services Department — adapted the structural competency framework beyond medical education for use in strategic planning, community organizing, and state government. Directly relevant to slide 15: this is the published work behind the "structural competency extended to Doña Ana County health councils and state agencies" footnote. Co-authored by John Andazola and Iván de la Rosa.

REPORT New Mexico Legislature, Legislative Finance Committee. (2023). *Addressing substance use disorders. State-level policy brief documenting NM's SUD landscape, funding, and gaps. Useful for understanding the policy context your work operates within, and for making the case for resources and systemic change internally.*

REPORT HRSA. *Enhancing rural access to medications for opioid use disorder.*
Federal report addressing the rural MOUD access gap. Relevant to La Clínica's service area and the geographic barriers patients face in reaching treatment. Useful for administrators and care coordinators.

REPORT Project ECHO, UNM. *Breaking the cycle: Expanding opioid treatment in New Mexico.*
Documents UNM's Project ECHO model for expanding MOUD access through provider mentorship and telementoring. Directly applicable if La Clínica is looking to expand provider capacity or connect with state-level training infrastructure.

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*Not a training you complete — a practice you
return to.*